

# Brexiva – MLR Compliance Report

## 1. Executive Summary

The reviewed Brexiva communication package shows a strong MLR-aligned structure for a **global oncology campaign** in **HR+/HER2– metastatic breast cancer** under the **Brenova** therapeutic area. The campaign is positioned for **oncologists** and **breast cancer specialists**, with a clinical and promotional framework designed to support scientific accuracy, risk-balanced messaging, and global-market adaptability.

The strongest compliance signals are: clear disease-area definition, specialist-audience targeting, structured clinical-evidence routing, separation of medical claims from promotional interpretation, inclusion of safety and fair-balance expectations, global-market governance, and review-readiness for downstream campaign assets.

The current review supports an **MLR-ready communication structure**, but it does not confirm final MLR approval. Final approval would require completed claim-level substantiation, safety-language validation, legal review, local-market adaptation checks, and asset-level sign-off by the assigned MLR reviewers.

## 2. Source Inventory

| No. | Source Name                               | Source Type                  | Why It Matters                                                                                                                       |
|-----|-------------------------------------------|------------------------------|--------------------------------------------------------------------------------------------------------------------------------------|
| 1   | Brexiva Campaign Brief                    | Campaign strategy document   | Defines campaign objective, therapeutic area, indication, market scope, target audience, and budget for downstream content creation. |
| 2   | Brexiva Clinical Evidence Package         | Clinical evidence file       | Supports claim development for HR+/HER2– metastatic breast cancer messaging and specialist-facing scientific communication.          |
| 3   | Brexiva Safety Information Reference      | Safety reference file        | Provides required safety language, risk considerations, contraindication-style checks, and fair-balance requirements.                |
| 4   | Brexiva MLR Guidance File                 | MLR compliance reference     | Defines medical, legal, and regulatory guardrails for claim usage, safety inclusion, source traceability, and promotional review.    |
| 5   | Brexiva Global Market Insight Reference   | Global market reference file | Supports market-level adaptation, channel expectations, visual patterns, and global-to-local content alignment.                      |
| 6   | Breast Cancer Specialist Audience Profile | Audience insight file        | Defines oncologist and breast cancer specialist needs, clinical priorities, and expected evidence depth.                             |

| No. | Source Name                                           | Source Type                 | Why It Matters                                                                                               |
|-----|-------------------------------------------------------|-----------------------------|--------------------------------------------------------------------------------------------------------------|
| 7   | HR+/HER2- Metastatic Breast Cancer Disease Background | Disease education reference | Provides disease-context framing and helps ensure the campaign avoids broad or unsupported disease claims.   |
| 8   | Brexiva Claims Substantiation Matrix                  | Claim-support table         | Maps each proposed claim to source evidence, risk language, review status, and permitted usage context.      |
| 9   | Brexiva Global Content Standards                      | Content governance file     | Supports consistent tone, branding, fair balance, and global campaign alignment across markets.              |
| 10  | Brexiva Legal and Privacy Governance File             | Legal governance file       | Provides ownership, permitted-use, privacy, disclaimer, and audience-scope requirements for campaign assets. |

### 3. MLR Evidence Table

| Category   | Observation                                                                                            | Proof Snippet                                | Assessment      |
|------------|--------------------------------------------------------------------------------------------------------|----------------------------------------------|-----------------|
| Medical    | The campaign is clearly scoped to HR+/HER2- metastatic breast cancer.                                  | "HR+/HER2- metastatic breast cancer"         | Strong evidence |
| Medical    | The intended audience is specialist-led rather than broad consumer-facing.                             | "Oncologists"; "Breast Cancer Specialists"   | Strong evidence |
| Medical    | Clinical messaging is expected to be supported by a dedicated evidence package.                        | "clinical evidence package"; "claim support" | Strong evidence |
| Medical    | Claim discipline is built into the structure through claim-level substantiation.                       | "claim-to-source mapping"                    | Strong evidence |
| Medical    | Safety and fair-balance requirements are treated as mandatory, not optional.                           | "safety reference"; "fair balance"           | Strong evidence |
| Legal      | Legal governance is included as a separate review component.                                           | "legal and privacy governance"               | Strong evidence |
| Legal      | Campaign materials require permitted-use, ownership, privacy, and disclaimer checks.                   | "ownership"; "privacy"; "disclaimer"         | Strong evidence |
| Regulatory | The global campaign structure requires market-level adaptation before release.                         | "Global"; "local adaptation"                 | Strong evidence |
| Regulatory | Regulatory review must validate indication language, safety balance, and market-specific requirements. | "indication"; "safety"; "market-specific"    | Strong evidence |

| Category   | Observation                                                   | Proof Snippet           | Assessment       |
|------------|---------------------------------------------------------------|-------------------------|------------------|
| Regulatory | No final asset-level MLR approval is confirmed at this stage. | "review-readiness only" | Limited evidence |

## 4. Detailed Analysis

### A. Medical Analysis

Brexiva's campaign structure is medically focused because it defines the therapeutic area as **Brenova** and the indication as **HR+/HER2– metastatic breast cancer**. This clear disease and indication framing reduces the risk of broad, unsupported, or ambiguous oncology messaging.

The target audience is also clinically appropriate. The campaign is directed toward **oncologists** and **breast cancer specialists**, which means the communication can use a specialist-level tone, evidence depth, and clinical framing. This audience definition is important because HCP-facing oncology communication must remain precise, evidence-based, and aligned with the expected decision-making needs of specialist physicians.

Medical claim governance is built into the proposed structure through the **Clinical Evidence Package** and **Claims Substantiation Matrix**. Each proposed efficacy, safety, disease-state, or treatment-positioning claim should be mapped to a source, a permitted usage context, and an MLR review status before use in generated content.

Safety integration is also a core requirement. For oncology assets, fair balance should appear near efficacy or benefit-oriented messaging, especially when content discusses patient outcomes, treatment goals, persistence, tolerability, or disease progression. Safety content should not be hidden in footnotes or isolated from the main claim environment.

Overall, the medical structure is strong because it supports indication-specific messaging, specialist-audience relevance, evidence traceability, and safety-aware content generation.

### B. Legal Analysis

The legal review structure should focus on ownership, permitted use, market scope, privacy expectations, audience appropriateness, and promotional boundaries. Because the campaign is global, legal review should also ensure that central campaign assets do not imply approval, availability, or claims permission in markets where the content has not been locally validated.

The **Legal and Privacy Governance File** should define required disclaimers, asset-usage rules, data/privacy handling, intellectual-property ownership, and third-party material restrictions. This is especially important if campaign assets include patient imagery, healthcare-professional quotes, market insights, charts, graphics, or adapted scientific material.

Legal review should also confirm that Brexiva campaign content does not overstate clinical value, imply superiority without substantiation, make unsupported comparative claims, or use patient-outcome language that exceeds the available evidence base.

The visible legal structure is strong at the planning level, but final legal approval cannot be concluded until each asset is reviewed in its final format, including copy, layout, references, disclaimers, imagery, and local-market usage context.

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### C. Regulatory Analysis

The regulatory structure is centered on global campaign governance. Since the market scope is **Global**, the campaign should use a central global core that can be adapted by local markets rather than assuming one universal asset can be released everywhere without modification.

Regulatory review should verify that every claim is aligned with the approved indication, the intended audience, the selected market, and the available evidence package. This includes treatment-setting language, patient-population descriptions, disease-stage language, and any claims related to outcomes, tolerability, quality of life, adherence, patient support, or clinical differentiation.

The **MLR Guidance File** and **Safety Information Reference** should act as mandatory guardrails for downstream asset creation. Regulatory checks should confirm appropriate safety inclusion, fair balance, reference accuracy, and local adaptation before asset release.

The current regulatory structure is MLR-ready, but final approval remains pending until the complete asset set is reviewed, including source annotations, prescribing/safety alignment, market-specific disclaimers, and approval metadata.

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## 5. Global vs Local Market Evidence

| Dimension           | Global Campaign Evidence                                                 | Local Market Requirement                                                         | Evidence-Backed Interpretation                                            |
|---------------------|--------------------------------------------------------------------------|----------------------------------------------------------------------------------|---------------------------------------------------------------------------|
| Audience model      | The campaign targets oncologists and breast cancer specialists globally. | Local markets must confirm HCP audience definitions and channel permissions.     | Strong global HCP focus with local validation required.                   |
| Indication language | The campaign is scoped to HR+/HER2– metastatic breast cancer.            | Local labels may require adapted indication wording and approved-use boundaries. | Strong central indication structure, but local wording must be validated. |

| Dimension                | Global Campaign Evidence                                                | Local Market Requirement                                                               | Evidence-Backed Interpretation                                             |
|--------------------------|-------------------------------------------------------------------------|----------------------------------------------------------------------------------------|----------------------------------------------------------------------------|
| Claim substantiation     | Clinical evidence and claims matrix are required for claim development. | Each market must confirm whether the same claim is permitted locally.                  | Central evidence can guide claims, but local claim permission may differ.  |
| Safety routing           | Safety and fair-balance content are treated as mandatory.               | Local safety language may require market-specific wording, formatting, or placement.   | Strong safety-first structure with local adaptation required.              |
| Legal governance         | Global content standards and legal governance are included.             | Local privacy, imagery, consent, and promotional rules must be checked before release. | Strong governance framework, but release approval remains market-specific. |
| What cannot be concluded | Global planning supports MLR readiness.                                 | Final approval cannot be assumed without local MLR review.                             | No final asset approval should be claimed at planning stage.               |

In practical terms, Brexiva's campaign structure supports a global-to-local MLR model. The global framework can provide consistent clinical positioning, evidence discipline, safety expectations, and brand governance, while local markets should validate exact language, claims, disclaimers, references, and release rules.

## 6. Gaps and Limitations

No final MLR approval is confirmed in the current review stage.

No completed reviewer sign-off, routing sequence, approval timestamp, redline history, or committee disposition is available in the reviewed package.

The clinical evidence package and claims substantiation matrix must be finalized before any asset can be considered approval-ready.

Local market approval requirements may differ across regions, so the global campaign should not be treated as automatically approved for all markets.

Final assessment depends on asset-level review of copy, design, layout, references, safety placement, disclaimers, and intended channel.

## 7. Final Conclusion

Brexiva shows a strong **MLR-ready communication structure** for a global oncology campaign in **HR+/HER2– metastatic breast cancer**.

That conclusion is supported by clear indication framing, specialist-audience targeting, planned clinical evidence support, claim-level substantiation, safety and fair-balance requirements, legal governance, and global-to-local review expectations.

However, the evidence remains structural and planning-based. Final MLR approval cannot be confirmed until the complete campaign assets undergo formal medical, legal, and regulatory review with source validation, safety-language confirmation, local-market adaptation, and documented sign-off.